

CEDAR FAMILY PRACTICE
Established Patient Visit

Patient: Wagner, Rebecca
DOB: 1967-11-29 MRN: CFP-0088776
Date of Service: 02/15/2023
Rendering Provider: Joseph Whitman, MD
Encounter Type: Established patient, moderate complexity (CPT 99214)
Plus: CPT 93000 – 12-lead ECG with interpretation
Claim Control Number: UHC-2024041902

CHIEF COMPLAINT

Intermittent palpitations and lightheadedness x 3 weeks.

HPI

55-year-old female presents with recurrent episodes of "fluttering" sensation in the chest, lasting 1-3 minutes, occurring 2-3 times weekly. Two episodes associated with brief lightheadedness without syncope. No exertional component. Reports increased caffeine intake recently and significant work stress. No known cardiac history. No chest pain, dyspnea, orthopnea, or edema.

EXAM / VITALS

BP 138/86 HR 78 (regular) SpO2 99% RA
CV: RRR, no murmurs/rubs/gallops, normal S1/S2.
Pulm: CTAB. No JVD. No peripheral edema.

DIAGNOSTICS PERFORMED TODAY

- 12-lead ECG (CPT 93000): normal sinus rhythm, rate 76, normal axis, no ST/T abnormalities, QTc 412 ms. Interpreted by Whitman, MD.

ASSESSMENT/PLAN

1. Palpitations, unspecified (R00.2). ECG unremarkable today.
2. Likely benign etiology given normal baseline ECG; rule out PSVT/PACs.
 - Ordered 14-day Zio patch (separate claim).
 - TSH, CBC, BMP today.
3. Advised reduction in caffeine intake. Stress management discussed.

E/M LEVEL: 99214 – established patient, moderate-complexity MDM
(new problem with workup ordered, prescription drug consideration).

RTC 2-3 weeks for monitor results.

Signed: Whitman, J. MD – 02/15/2023 14:33

BILLING NOTE (re: timely filing CO-29 denial):

Both line items on this claim were on the same date of service.
Original 837P submitted 02/17/2023. Resubmission 06/03/2023 corrected a payer-id mismatch identified by clearinghouse. Timely filing window should be measured from the original submission date.